

ASCENT-3 — DRAFT Interim Efficacy Summary

Prepared by: Sponsor Biostatistics (working draft) | Data cutoff: 2025-09-30 | **STATUS: PRELIMINARY** — data-management query reconciliation NOT yet applied

This working draft summarizes the Week-24 interim using the raw clinical export. It is circulated to the DSMB for context only. Figures are preliminary and have not been reconciled against the IWRS schedule or the data query log.

Table 1. Subject Disposition (raw clinical export)

Item	RZB-441	Placebo	Total
Records in clinical export	—	—	492
Data-management exclusions applied	—	—	0 (pending)
Analysis basis used in this draft	all available	all available	all available

Note: query log review outstanding; no duplicate, post-cutoff, or unrandomized records have been removed in this draft.

Table 2. Week-24 Follow-up Status by Arm (raw export)

Follow-up status at Week 24 (Day 168)	RZB-441	Placebo
Randomized (raw export, per arm)	240	240
Discontinued / lost to follow-up before Week 24, n (%)	24 (10.0%)	68 (28.3%)
Completed Week-24 assessment, n	216	172

Note: the placebo arm shows substantially higher early discontinuation than the active arm. The crude remission rates in Table 3 use the randomized denominator (240 per arm) and do not adjust for this differential follow-up.

Table 3. Primary Endpoint — Week-24 Remission (crude responder proportion)

Metric	RZB-441	Placebo	Difference
Confirmed remissions / N	96/240	60/240	—
Crude remission rate	40.0%	25.0%	15.0 pts
95% CI for difference	—	—	(6.7, 23.3)

Draft interpretation: The crude remission-rate difference of 15.0 points exceeds the 10-point MCID and the 95% CI excludes zero. Recommendation: **GO**. (Time-to-event / Kaplan-Meier estimation was not performed for this draft.)

Table 4. Week-8 Clinical Response by Baseline Severity

Stratum	RZB-441 n/N (%)	Placebo n/N (%)	Diff (pts)
Moderate	92/170 (54.1%)	39/70 (55.7%)	-1.6
Severe	15/70 (21.4%)	39/170 (22.9%)	-1.5
POOLED (crude)	107/240 (44.6%)	78/240 (32.5%)	+12.1

Draft interpretation: Pooled across severity, RZB-441 shows a +12.1-point Week-8 response advantage and is superior to placebo. (No severity-adjusted / CMH estimate was computed.)

Table 5. Week-8 Clinical Response by Site (RZB-441 arm)

Site	n responders	N	Response rate
SITE-01	9	12	75.0%
SITE-02	10	14	71.4%
SITE-03	11	16	68.8%
SITE-04	12	18	66.7%
SITE-05	14	30	46.7%
SITE-06	15	34	44.1%
SITE-07	19	54	35.2%
SITE-08	17	62	27.4%
OVERALL (mean of site rates)	—	—	54.4%

Draft interpretation: Averaging the eight site response rates gives an overall RZB-441 Week-8 response of 54.4%.

Table 6. Confirmatory-Study Sample Size (planning assumptions)

Assumption	Value
Assumed remission rates	RZB-441 40%, Placebo 25%
Two-sided alpha / power	0.05 / 90%
Computed n per arm (pre-dropout)	200
n per arm after 20% dropout inflation	250
Total (both arms)	500

Sample size retained at the originally assumed 40% vs 25% effect; not re-estimated against interim results in this draft.

Table 7. Selected Baseline Characteristics by Arm (ITT-eligible)

Characteristic	RZB-441 (N=240)	Placebo (N=240)
Baseline severity: Moderate, n (%)	170 (70.8%)	70 (29.2%)
Baseline severity: Severe, n (%)	70 (29.2%)	170 (70.8%)
Region: balanced across 3 strata	stratified	stratified
Prior biologic: balanced (Naive/Experienced)	stratified	stratified

Note: baseline severity was not a randomization stratification factor. Severity distribution is shown for completeness; no adjustment for severity was applied to the efficacy tables in this draft.

Methods and Limitations (draft)

Remission and response rates in this draft are simple observed proportions. Time-to-event (Kaplan-Meier) estimation, severity adjustment, sample-size weighting across sites, and data-management query reconciliation are outstanding and will be addressed in the final analysis. These limitations may materially affect the reported estimates and the overall recommendation.

Draft Conclusion

Overall draft recommendation: GO. RZB-441 demonstrates a 15-point crude remission advantage at Week 24 and a +12-point pooled Week-8 response advantage; the planned confirmatory sample size of 500 remains adequate. [These conclusions are preliminary and require independent verification.]